GOVCARE EHR

CKD AI CLINICAL DECISION SUPPORT REPORT

Clinical decision support only - clinician review required

PATIENT & REPORT DETAILS

Patient	Demo Patient H02 0002
Patient number	PAT-NET-H02-0002
DOB	1945-07-14
Sex	female
Assessment ID	4d44f546-2886-45cf-91f9-60af9704df9a
Assessment time	2026-10-13 09:54:01.846547
Hospital	GovCare Demo Hospital 02
Department	Not recorded
Clinician / operator	Local Super Admin
Model	GOVCARE-CKD-LOGISTIC-1.0

CKD RISK SUMMARY

Predicted CKD risk	Risk category	AI confidence
100.0%	**URGENT REVIEW**	**100.0%**

CKD CLASSIFICATION & CLINICAL CODING

Suggested CKD classification	Stage 3b
G category	G3b
Suggested ICD code	N18.32
Clinical description	Chronic kidney disease, stage 3b
Terminology system	http://hl7.org/fhir/sid/icd-10-cm
Basis	Reviewed eGFR 32 mL/min/1.73m²; chronicity and diagnosis require clinician confirmation
Coding status	AI_ASSISTED_SUGGESTION
Confirmation	Clinician confirmation required

A single eGFR result does not independently establish chronic kidney disease or chronicity. N18.6 (ESRD) is not inferred from eGFR alone.

○ G1	○ G2	○ G3a	● G3b	○ G4	○ G5

RENAL & ASSOCIATED FINDINGS

Parameter	Result	Unit	Source
eGFR	32	mL/min/1.73m²	MANUAL
Serum creatinine	30	mg/dL	MANUAL
Urine albumin/creatinine ratio (ACR)	32	mg/g	MANUAL
Urine protein	32	mg/dL	MANUAL
Systolic blood pressure	280	mmHg	MANUAL

Diastolic blood pressure	32	mmHg	MANUAL
HbA1c	3	%	MANUAL
BMI	32	kg/m²	MANUAL
Serum potassium	3	mmol/L	MANUAL
Hemoglobin	3	g/dL	MANUAL

CKD-RELATED CLINICAL RISKS & PREDICTIVE INTELLIGENCE

Sri Lankan Longitudinal Renal & Hemodialysis Data

Saved snapshot: ac4859cc-4d85-48f1-a3ce-efb2a656c2e5 | Evaluated 2026-10-13T04:24:02.157053400Z. Export reads saved clinical evidence; no inference is performed.

CKD Progression Risk

CALCULATED | High CKD progression monitoring signal

Clinical Data Readiness: 100% Ready | 9/9 parameters | Missing: 0

Monitoring level: High

Qualitative measurement-based monitoring band; no future-event probability or forecast horizon is estimated.

Why: Reduced eGFR: 32.00 mL/min/1.73 m2.

Why: Elevated ACR: 32.00 mg/g.

Why: Serum creatinine: 30.00 mg/dL; interpret against the patient's source reference range.

Single clinical snapshot available. Longitudinal trend requires previous dated observations.

Measurement-based monitoring; no new disease probability or confirmed diagnosis. Source details, rules and clinical recommendations follow.

Kidney Failure Risk Indicator

CALCULATED | Kidney failure risk indicator calculated

Clinical Data Readiness: 80% Ready | 4/5 parameters | Missing: 1

Monitoring level: Below high-signal threshold

High monitoring signal requires eGFR <15 or verified chronic dialysis; creatinine alone cannot establish G5D.

Why: eGFR 32.00 mL/min/1.73 m2 does not meet the <15 high-signal threshold.

Dialysis verification required

Needs: Verified dialysis status

Measurement-based monitoring; no new disease probability or confirmed diagnosis. Source details, rules and clinical recommendations follow.

Hyperkalemia Monitoring

CALCULATED | Below hyperkalemia monitoring threshold

Clinical Data Readiness: 100% Ready | 1/1 parameters | Missing: 0

Measured: 3 mmol/L | Undated snapshot; time not verified

Monitoring level: Normal

Current measurement monitoring is independent of the maintenance-HD research forecast. The low band does not exclude hypokalemia or other abnormalities.

Why: Serum potassium 3.00 mmol/L; configured bands: <5.0, 5.0-5.5 inclusive, >5.5.

Separate HD research forecast: Eligible dialysis context not verified

Measurement-based monitoring; no new disease probability or confirmed diagnosis. Source details, rules and clinical recommendations follow.

Anemia / Hb Deterioration

CALCULATED | Anemia monitoring signal

Clinical Data Readiness: 100% Ready | 1/1 parameters | Missing: 0

Measured: 3 g/dL | Undated snapshot; time not verified

Monitoring level: High

Measured Hb monitoring and decline rate are separate from the installed persistence baseline; the baseline is not an anemia prognosis.

Why: Hemoglobin 3.00 g/dL; configured bands: <10, 10-12 inclusive, >12. These are monitoring bands, not universal normal ranges or an anemia diagnosis.

Separate HD research forecast: Eligible dialysis context not verified

Measurement-based monitoring; no new disease probability or confirmed diagnosis. Source details, rules and clinical recommendations follow.

Dialysis Adequacy / URR

INSUFFICIENT DATA | Dialysis session measurements required

Clinical Data Readiness: 0% Ready | 0/3 parameters | Missing: 3

Requires hemodialysis session data

Needs: Pre-HD Urea

Needs: Post-HD Urea

Needs: Dialysis date

Separate HD research forecast: Eligible dialysis context not verified

Measurement-based monitoring; no new disease probability or confirmed diagnosis. Source details, rules and clinical recommendations follow.

AKI Risk / Creatinine Trend

PARTIALLY CALCULATED | AKI trend not calculated

Clinical Data Readiness: 25% Ready | 1/4 parameters | Missing: 3

Baseline creatinine required for AKI trend calculation

Needs: Previous creatinine

Needs: Current creatinine measurement date

Needs: Previous creatinine measurement date

Measurement-based monitoring; no new disease probability or confirmed diagnosis. Source details, rules and clinical recommendations follow.

Clinical Decision Support / Research Use. Outputs require clinician interpretation. Research-model outputs are not independently validated diagnoses.

Prediction generated from available clinical measurements. Longitudinal forecasting requires repeated dated observations. Clinical data readiness is input completeness, not diagnostic certainty or disease probability.

SIX-MODULE EVIDENCE & CLINICIAN REVIEW

CKD Progression / Renal Trajectory

CALCULATED - High CKD progression monitoring signal

Mapping rule: GOVCARE-CKD-FEATURE-MAPPING-1.0 | Transparent snapshot monitoring rule

GovCare qualitative eGFR/ACR grid: High if eGFR <45, ACR >300, or eGFR <60 with ACR ≥30; Moderate if eGFR <60 or ACR ≥30; Low requires eGFR ≥60 AND ACR <30. Categories condense high/very-high monitoring; not a validated progression model.
Serum Creatinine: 30 mg/dL | Undated assessment snapshot | Source: CKD_ASSESSMENT / 4d44f546-2886-45cf-91f9-60af9704df9a
eGFR: 32 mL/min/1.73m² | Undated assessment snapshot | Source: CKD_ASSESSMENT / 4d44f546-2886-45cf-91f9-60af9704df9a
Protein in urine: 32 mg/dL | Undated assessment snapshot | Source: CKD_ASSESSMENT / 4d44f546-2886-45cf-91f9-60af9704df9a
Urine ACR: 32 mg/g | Undated assessment snapshot | Source: CKD_ASSESSMENT / 4d44f546-2886-45cf-91f9-60af9704df9a
Age: 81 years | Undated assessment snapshot | Source: CKD_ASSESSMENT / 4d44f546-2886-45cf-91f9-60af9704df9a
Systolic Blood Pressure: 280 mmHg | Undated assessment snapshot | Source: CKD_ASSESSMENT / 4d44f546-2886-45cf-91f9-60af9704df9a
Diastolic Blood Pressure: 32 mmHg | Undated assessment snapshot | Source: CKD_ASSESSMENT / 4d44f546-2886-45cf-91f9-60af9704df9a
Fasting Blood Sugar: 320 mg/dL | Undated assessment snapshot | Source: CKD_ASSESSMENT / 4d44f546-2886-45cf-91f9-60af9704df9a
HbA1c: 3 % | Undated assessment snapshot | Source: CKD_ASSESSMENT / 4d44f546-2886-45cf-91f9-60af9704df9a

Clinical recommendations

Clinician review of eGFR, ACR, urine findings and renal chronicity; obtain previous dated observations for longitudinal forecasting.
Why: Reduced eGFR: 32.00 mL/min/1.73 m2.
Why: Elevated ACR: 32.00 mg/g.
Why: Serum creatinine: 30.00 mg/dL; interpret against the patient's source reference range.
Why: HbA1c / glucose are supporting glycemic measurements; a diabetes diagnosis is not inferred.
Available: Serum Creatinine | 1/1 | 30.00 mg/dL; undated snapshot only
Available: eGFR | 1/1 | 32.00 mL/min/1.73m²; undated snapshot only
Available: Protein in urine | 1/1 | 32.00 mg/dL; undated snapshot only
Available: Urine ACR | 1/1 | 32.00 mg/g; undated snapshot only
Available: Age | 1/1 | 81.00 years; undated snapshot only
Available: Systolic Blood Pressure | 1/1 | 280.00 mmHg; undated snapshot only
Available: Diastolic Blood Pressure | 1/1 | 32.00 mmHg; undated snapshot only
Available: Fasting Blood Sugar | 1/1 | 320.00 mg/dL; undated snapshot only
Available: HbA1c | 1/1 | 3.00 %; undated snapshot only

Next action: Add historical laboratory data

Directions compare the two latest valid values per domain using a 1% neutral band; this is not a prognosis or a G1-to-G5 forecast.
Dialysis timing, clinical context and measurement uncertainty require clinician interpretation.
Clinical feature sources: 1 | Separately stored renal / HD observations: 0 | Reliability: INSUFFICIENT | Horizon: No probability horizon | Last run: Not recorded

Kidney Failure / G5D

CALCULATED - Kidney failure risk indicator calculated

Mapping rule: GOVCARE-CKD-FEATURE-MAPPING-1.0 | Clinical evidence / monitoring rule

eGFR: 32 mL/min/1.73m² | Undated assessment snapshot | Source: CKD_ASSESSMENT / 4d44f546-2886-45cf-91f9-60af9704df9a
Serum Creatinine: 30 mg/dL | Undated assessment snapshot | Source: CKD_ASSESSMENT / 4d44f546-2886-45cf-91f9-60af9704df9a
Protein in urine: 32 mg/dL | Undated assessment snapshot | Source: CKD_ASSESSMENT / 4d44f546-2886-45cf-91f9-60af9704df9a
Urine ACR: 32 mg/g | Undated assessment snapshot | Source: CKD_ASSESSMENT / 4d44f546-2886-45cf-91f9-60af9704df9a

Clinical recommendations

Review renal function and chronicity; verify dialysis status before G5D labeling or maintenance-HD model use.
Why: eGFR 32.00 mL/min/1.73 m2 does not meet the <15 high-signal threshold.
Available: eGFR | 1/1 | 32.00 mL/min/1.73m²; undated snapshot only
Available: Serum Creatinine | 1/1 | 30.00 mg/dL; undated snapshot only
Available: Protein in urine | 1/1 | 32.00 mg/dL; undated snapshot only
Available: Urine ACR | 1/1 | 32.00 mg/g; undated snapshot only
Needs review / data: Verified dialysis status | 0/1 | Dialysis verification required

Next action: Verify dialysis status

High creatinine alone does not establish G5D. This card reports recorded clinical evidence, not a diagnosis generated by AI.
No 2-year or 5-year kidney-failure probability is calculated.
Clinical feature sources: 1 | Separately stored renal / HD observations: 0 | Reliability: INSUFFICIENT | Horizon: No probability horizon | Last run: Not recorded

Hyperkalemia Monitoring

CALCULATED - Below hyperkalemia monitoring threshold

Mapping rule: GOVCARE-CKD-FEATURE-MAPPING-1.0 | Current potassium monitoring rule

Serum Potassium: 3 mmol/L | Undated assessment snapshot | Source: CKD_ASSESSMENT / 4d44f546-2886-45cf-91f9-60af9704df9a

Clinical recommendations

Review measured potassium with the clinician, source quality, medications and sampling context; use true paired pre/post-HD samples when available.
Why: Serum potassium 3.00 mmol/L; configured bands: <5.0, 5.0-5.5 inclusive, >5.5.
Available: Serum Potassium | 1/1 | 3.00 mmol/L; undated snapshot only

Next action: Add historical potassium result

Research outputs require clinician interpretation; internal evaluation is not independent clinical validation.
Dataset permission status remains UNVERIFIED.
This installed model is scoped to verified maintenance hemodialysis. No applicable dialysis context is recorded.
Requires two fresh verified HD observations 14–62 days apart, a current target result and five supported predictor values.
Clinical feature sources: 1 | Separately stored renal / HD observations: 0 | Reliability: INSUFFICIENT | Horizon: No probability horizon | Last run: Not recorded

Anemia / Hb Deterioration

CALCULATED - Anemia monitoring signal

Mapping rule: GOVCARE-CKD-FEATURE-MAPPING-1.0 | Current hemoglobin monitoring rule

Hemoglobin: 3 g/dL | Undated assessment snapshot | Source: CKD_ASSESSMENT / 4d44f546-2886-45cf-91f9-60af9704df9a

Clinical recommendations

Clinician review against age/sex-specific laboratory ranges, iron studies and clinical context; add a previous dated Hb result for deterioration monitoring.
Why: Hemoglobin 3.00 g/dL; configured bands: <10, 10-12 inclusive, >12. These are monitoring bands, not universal normal ranges or an anemia diagnosis.
Available: Hemoglobin | 1/1 | 3.00 g/dL; undated snapshot only

Next action: Add historical Hb result

Research outputs require clinician interpretation; internal evaluation is not independent clinical validation.
Dataset permission status remains UNVERIFIED.
Learned candidates failed the gate against persistence. Carries forward the last measured Hb; no anemia-classifier probability is installed.
This installed model is scoped to verified maintenance hemodialysis. No applicable dialysis context is recorded.
Requires two fresh verified HD observations 14–62 days apart, a current target result and five supported predictor values.
Clinical feature sources: 1 | Separately stored renal / HD observations: 0 | Reliability: INSUFFICIENT | Horizon: No probability horizon | Last run: Not recorded

Dialysis Adequacy / URR

INSUFFICIENT DATA - Dialysis session measurements required

Mapping rule: GOVCARE-CKD-FEATURE-MAPPING-1.0 | Measured same-session urea reduction ratio

URR = ((Pre-HD Urea − Post-HD Urea) / Pre-HD Urea) × 100

Clinical recommendations

Review measured adequacy with the clinician; record actual pre-HD urea, post-HD urea and dialysis date. BUN from the CKD form is not a paired dialysis urea measurement.
Needs review / data: Pre-HD Urea | 0/1 | Requires hemodialysis session data
Needs review / data: Post-HD Urea | 0/1 | Requires hemodialysis session data
Needs review / data: Dialysis date | 0/1 | Actual dialysis session date; upload time is not substituted

Next action: Add dialysis-session data

Research outputs require clinician interpretation; internal evaluation is not independent clinical validation.
Dataset permission status remains UNVERIFIED.
This installed model is scoped to verified maintenance hemodialysis. No applicable dialysis context is recorded.
Requires two fresh verified HD observations 14–62 days apart, a current target result and five supported predictor values.
Clinical feature sources: 0 | Separately stored renal / HD observations: 0 | Reliability: INSUFFICIENT | Horizon: No probability horizon | Last run: Not recorded

AKI / Renal Clinical Deterioration

PARTIALLY CALCULATED - AKI trend not calculated

Mapping rule: GOVCARE-CKD-FEATURE-MAPPING-1.0 | Dated creatinine change / acute review rule

Creatinine-based clinical review only: rise ≥0.3 mg/dL within 48 h or ≥1.5 times a dated baseline within 7 days. No AKI staging, urine-output assessment or diagnosis is generated.

Creatinine change (%) = ((Current Creatinine − Previous Creatinine) / Previous Creatinine) × 100

Serum Creatinine: 30 mg/dL | Undated assessment snapshot | Source: CKD_ASSESSMENT / 4d44f546-2886-45cf-91f9-60af9704df9a

Clinical recommendations

Obtain previous creatinine with its actual measurement date; clinician review of timing, acute illness, medications and urine output is required.
Available: Serum Creatinine | 1/1 | 30.00 mg/dL; undated snapshot only
Needs review / data: Previous creatinine | 0/1 | Baseline creatinine required for AKI trend calculation
Needs review / data: Current creatinine measurement date | 0/1 | Current and previous results need distinct actual dates
Needs review / data: Previous creatinine measurement date | 0/1 | A recorded baseline date is required

Next action: Add baseline creatinine history

Directional composite – not a validated prognosis score.
Equal weights; two latest valid observations per domain; 1% neutral stability band. Missing domains are excluded, never scored as zero.
A directional percentage is not a disease probability.
Clinical feature sources: 1 | Separately stored renal / HD observations: 0 | Reliability: INSUFFICIENT | Horizon: No probability horizon | Last run: Not recorded

PREDICTIVE CLINICAL INTELLIGENCE - LONGITUDINAL REVIEW

Live authorized EHR history at report generation time. Projected values are statistical extrapolations and are never stored or presented as laboratory observations.

Recorded G / A category	Latest eGFR	Latest creatinine	Data completeness
- / -	Unavailable	Unavailable	Unavailable

HISTORICAL eGFR & PROJECTED TRAJECTORY

No usable dated eGFR history is available.

Horizon	Current	Projected	Projected range	Trend	Reliability	Status / interval method
3 months	-	Unavailable	Not estimated	UNAVAILABLE	LOW	INSUFFICIENT DATA NOT ESTIMATED
6 months	-	Unavailable	Not estimated	UNAVAILABLE	LOW	INSUFFICIENT DATA NOT ESTIMATED
12 months	-	Unavailable	Not estimated	UNAVAILABLE	LOW	INSUFFICIENT DATA NOT ESTIMATED

EXPLAINABILITY & DATA-QUALITY LIMITATIONS

Renal safety: INSUFFICIENT DATA. Renal observations have unresolved date, unit, provenance, plausibility or conflicting-result issues. Reconcile the source records before forecasting. A usable eGFR result from the previous 90 days is unavailable. A usable creatinine result from the previous 90 days is unavailable. At least two dated eGFR measurements spanning 90 days are required by the longitudinal sufficiency policy. An active or recent admission requires acute illness and fluid-status review before long-term forecasting (GovCare 14-day context policy).
- MISSING CLINICAL DATE: A usable measurement date is required; upload time is never substituted.
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Medication context: No recent active prescription record was found. Medication use and nephrotoxic exposure are unknown; reconcile with the patient.

CLINICIAN-CONTROLLED RECOMMENDATIONS

- No dated eGFR available | AWAITING REVIEW | Confirm whether a newer result exists and whether repeat measurement is clinically appropriate.
- No dated Urine ACR available | AWAITING REVIEW | Confirm whether a newer result exists and whether repeat measurement is clinically appropriate.

Method: TREND PROJECTION · Model: trend-linear-2.0 · Validation: STATISTICAL_PROJECTION_NOT_CLINICALLY_VALIDATED · Local validation: NOT_APPLICABLE

Clinical decision support only. Forecasts and risk estimates are not autonomous diagnoses.

CLINICAL DATA USED FOR ASSESSMENT

Parameter	Result	Unit	Source	Coding
Urine albumin/creatinine ratio (ACR)	32	mg/g	MANUAL	-

Age	81	-	CALCULATED	-
BMI	32	kg/m²	MANUAL	-
Blood urea nitrogen (BUN)	32	mg/dL	MANUAL	-
Diastolic blood pressure	32	mmHg	MANUAL	-
Edema	1	-	MANUAL	-
Family history of kidney disease	0	-	MANUAL	-
Fasting blood sugar	320	mg/dL	MANUAL	-
eGFR	32	mL/min/1.73m²	MANUAL	-
HbA1c	3	%	MANUAL	-
Hemoglobin	3	g/dL	MANUAL	-
NSAID use	1	-	MANUAL	-
Previous acute kidney injury	1	-	MANUAL	-
Urine protein	32	mg/dL	MANUAL	-
Serum creatinine	30	mg/dL	MANUAL	-
Serum potassium	3	mmol/L	MANUAL	-
Systolic blood pressure	280	mmHg	MANUAL	-

KEY FACTORS IN THIS MODEL OUTPUT

Contribution direction describes this model's prediction, not whether a clinical value is healthy, safe or protective.

Factor	Value	Unit	Direction	Contribution
Serum creatinine	30	mg/dL	Increases prediction	16.5493
Urine protein	32	mg/dL	Increases prediction	8.2157
Systolic blood pressure	280	mmHg	Increases prediction	2.2091
Fasting blood sugar	320	mg/dL	Increases prediction	1.8731
Hemoglobin	3	g/dL	Reduces prediction	-0.9645
eGFR	32	mL/min/1.73m²	Increases prediction	0.7945
Diastolic blood pressure	32	mmHg	Reduces prediction	-0.6470
Edema	1	-	Increases prediction	0.5688
HbA1c	3	%	Reduces prediction	-0.5590
Serum potassium	3	mmol/L	Reduces prediction	-0.2160
Urine albumin/creatinine ratio (ACR)	32	mg/g	Increases prediction	0.1610
Blood urea nitrogen (BUN)	32	mg/dL	Increases prediction	0.1281

RECOMMENDED CLINICAL REVIEW

1. Prompt clinician review is recommended.
2. Correlate with current renal function, urine findings, and clinical examination.

CLINICIAN REVIEW

Suggested classification	Stage 3b
Suggested code	N18.32
Coding decision	[ ] Confirm [ ] Modify [ ] Reject

Final clinician-selected code	_____
Clinical notes	_____ _____
Reviewed by / date	_____

This output does not diagnose disease or replace professional medical judgement. AI-assisted coding must be reviewed before a confirmed FHIR Condition is created.